

Rosuvuid Tablet

eVIROS02-var1 (MY)

DESCRIPTION

Rosuvuid Tablet 5mg: Round, 6.0mm in diameter, yellow film-coated tablet with deep convex faces.

Rosuvuid Tablet 10mg: Round, 7.0mm in diameter, pink film-coated tablet, bevel-edged, shallow convex faces, "H" and "D" embossed on each side separated with a score line on the same face.

Rosuvuid Tablet 20mg: Round, 9.0mm in diameter, pink film-coated tablet, bevel-edged and shallow convex faces.

COMPOSITION

Rosuvuid Tablet 5mg: Each tablet contains Rosuvastatin Calcium equivalent to Rosuvastatin 5 mg

Rosuvuid Tablet 10mg: Each tablet contains Rosuvastatin Calcium equivalent to Rosuvastatin 10 mg

Rosuvuid Tablet 20mg: Each tablet contains Rosuvastatin Calcium equivalent to Rosuvastatin 20 mg

PHARMACODYNAMICS

Rosuvastatin is a selective and competitive inhibitor of HMG-CoA reductase, the rate-limiting enzyme that converts 3-hydroxy-3-methylglutaryl coenzyme A to mevalonate, a precursor for cholesterol. The primary site of action of rosuvastatin is the liver, the target organ for cholesterol lowering. Rosuvastatin increases the number of hepatic LDL receptors on the cell-surface, enhancing uptake and catabolism of LDL and it inhibits the hepatic synthesis of VLDL, thereby reducing the total number of VLDL and LDL particles. Rosuvastatin reduces elevated LDL-cholesterol, total cholesterol and triglycerides and increases HDL-cholesterol. It also lowers ApoB, nonHDL-C, VLDL-C, VLDL-TG and increases ApoA-I. Rosuvastatin also lowers the LDL-C/HDL-C, total C/HDL-C and nonHDL-C/HDL-C and the ApoB/ApoA-I ratios.

PHARMACOKINETICS

Absorption: Maximum rosuvastatin plasma concentrations are achieved approximately 5 hours after oral administration. The absolute bioavailability is approximately 20%.

Distribution: Rosuvastatin is taken up extensively by the liver which is the primary site of cholesterol synthesis and LDL-C clearance. The volume of distribution of rosuvastatin is approximately 134 L. Approximately 90% of rosuvastatin is bound to plasma proteins, mainly to albumin.

Metabolism: Rosuvastatin undergoes limited metabolism (approximately 10%). In vitro metabolism studies using human hepatocytes indicate that rosuvastatin is a poor substrate for cytochrome P450-based metabolism. CYP2C9 was the principal isoenzyme involved, with 2C19, 3A4 and 2D6 involved to a lesser extent. The main metabolites identified are the N-desmethyl and lactone metabolites. The N-desmethyl metabolite is approximately 50% less active than rosuvastatin whereas the lactone form is considered clinically inactive. Rosuvastatin accounts for greater than 90% of the circulating HMG-CoA reductase inhibitor activity.

Excretion: Approximately 90% of the rosuvastatin dose is excreted unchanged in the faeces (consisting of absorbed and non-absorbed active substance) and the remaining part is excreted in urine. Approximately 5% is excreted unchanged in urine. The plasma elimination half-life is approximately 19 hours. The elimination half-life does not increase at higher doses. The geometric mean plasma clearance is approximately 50 litres/hour (coefficient of variation 21.7%). As with other HMG-CoA reductase inhibitors, the hepatic uptake of rosuvastatin involves the membrane transporter OATP-C. This transporter is important in the hepatic elimination of rosuvastatin.

Linearity: Systemic exposure of rosuvastatin increases in proportion to dose. There are no changes in pharmacokinetic parameters following multiple daily doses.

INDICATIONS

Rosuvastatin is indicated as an adjunct to diet, at least equivalent to the Adult Treatment Panel III (ATP III TLC diet), for the reduction of elevated total cholesterol, LDL-cholesterol, ApoB, the total cholesterol: HDL-cholesterol ratio and triglycerides and for increasing HDL-C, in hyperlipidemic and dyslipidemic conditions, when response to diet and exercise alone has been inadequate including:

Prevention of Cardiovascular Events

In adult patients with an increased risk of atherosclerotic cardiovascular disease based on the presence of cardiovascular disease risk markers such as an elevated hsCRP level, age, hypertension, low HDL-C, smoking or a family history of premature coronary heart disease, Rosuvastatin is indicated to reduce total mortality and the risk of major cardiovascular events (cardiovascular death, stroke, MI, unstable angina, or arterial revascularization)

Rosuvastatin is indicated as an adjunct to diet for the treatment of patients with primary dysbetalipoproteinemia (Type III Hyperlipoproteinemia).

Primary hypercholesterolaemia (Type IIa including heterozygous familial hypercholesterolaemia and severe non-familial hypercholesterolaemia)

Combined (mixed) dyslipidemia (Type IIb)

Homozgous familial hypercholesterolaemia where Rosuvastatin is used either alone or as an adjunct to diet and other lipid lowering treatment such as apheresis.

Rosuvastatin is indicated as adjunctive therapy to diet to slow the progression of atherosclerosis in adult patients as part of a treatment strategy to lower Total-C and LDL-C to target levels.

Pediatric Patients 10 to 17 years of age with Heterozygous Familial Hypercholesterolemia (HeFH): Adjunct to diet to reduce Total-C, LDL-C and ApoB levels in adolescent boys and girls, who are at least one year post-menarche, 10-17 years of age with heterozygous familial hypercholesterolemia if after an adequate trial of diet therapy the following findings are present: LDL-C > 190 mg/dL or > 160mg/dL and there is a positive family history of premature cardiovascular disease (CVD) or two or more other CVD risk factors.

CONTRAINDICATIONS

Rosuvuid Tablet is contraindicated:

- in patients with hypersensitivity to rosuvastatin or to any of the excipients.
- in patients with active liver disease including unexplained, persistent elevations of serum transaminases and any serum transaminase elevation exceeding 3 x the upper limit of normal (ULN).
- in patients with severe renal impairment (creatinine clearance <30 ml/min).
- in patients with myopathy.
- in patients receiving concomitant ciclosporin.
- during pregnancy and lactation and in women of childbearing potential not using appropriate contraceptive measures.

WARNINGS AND PRECAUTIONS

Skeletal Muscle Effects: Gemfibrozil increases the risk of myopathy when given concomitantly with some HMG-CoA reductase inhibitors. Therefore, the combination of rosuvastatin and gemfibrozil is not recommended. The benefit of further alterations in lipid levels by the combined use of rosuvastatin with fibrates or niacin should be carefully weighed against the potential risks of such combinations.

Effects on skeletal muscle e.g. myalgia, myopathy and, rarely, rhabdomyolysis have been reported in rosuvastatin-treated patients with all doses and in particular with doses > 20 mg. Very rare cases of rhabdomyolysis with the use of ezetimibe in combination with HMG-CoA reductase inhibitors. A pharmacodynamic interaction cannot be excluded and caution should be exercised with their combined use. As with other HMG-CoA reductase inhibitors, the reporting rate for rhabdomyolysis associated with rosuvastatin is higher at the 40mg dose.

- **Creatine Kinase Measurement:** Creatine Kinase (CK) should not be measured following strenuous exercise or in the presence of a plausible alternative cause of CK increase which may confound interpretation of the result. If CK levels are significantly elevated at baseline (>5xULN) a confirmatory test should be carried out within 5-7 days. If the repeat test confirms a baseline CK >5xULN, treatment should not be started.
- **Before Treatment:** Rosuvastatin, as with other HMG-CoA reductase inhibitors, should be prescribed with caution in patients with pre-disposing factors for myopathy/ rhabdomyolysis. Such factors include:
 - renal impairment
 - hypothyroidism

- personal or family history of hereditary muscular disorders
- previous history of muscular toxicity with another HMG-CoA reductase inhibitor or fibrate
- alcohol abuse
- age >70 years
- situations where an increase in plasma levels may occur
- concomitant use of fibrates.

In such patients the risk of treatment should be considered in relation to possible benefit and clinical monitoring is recommended. If CK levels are significantly elevated at baseline (>5xULN) treatment should not be started.

- **Whilst on Treatment:** Patients should be asked to report inexplicable muscle pain, weakness or cramps immediately, particularly if associated with malaise or fever. CK levels should be measured in these patients. Therapy should be discontinued if CK levels are markedly elevated (>5xULN) or if muscular symptoms are severe and cause daily discomfort (even if CK levels are ≤ 5x ULN). If symptoms resolve and CK levels return to normal, then consideration should be given to re-introducing rosuvastatin or an alternative HMG-CoA reductase inhibitor at the lowest dose with close monitoring. Routine monitoring of CK levels in asymptomatic patients is not warranted. There have been very rare reports of an immune-mediated necrotising myopathy (IMNM) during or after treatment with statins, including rosuvastatin. IMNM is clinically characterised by proximal muscle weakness and elevated serum creatine kinase, which persist despite discontinuation of statin treatment.

An increase in the incidence of myositis and myopathy has been seen in patients receiving other HMG-CoA reductase inhibitors together with fibric acid derivatives including gemfibrozil, ciclosporin, nicotinic acid, azole antifungals, protease inhibitors and macrolide antibiotics. Gemfibrozil increases the risk of myopathy when given concomitantly with some HMG-CoA reductase inhibitors. Therefore, the combination of rosuvastatin and gemfibrozil is not recommended. The benefit of further alterations in lipid levels by the combined use of rosuvastatin with fibrates or niacin should be carefully weighed against the potential risks of such combinations.

Rosuvastatin should not be used in any patient with an acute, serious condition suggestive of myopathy or predisposing to the development of renal failure secondary to rhabdomyolysis (e.g. sepsis, hypotension, major surgery, trauma, severe metabolic, endocrine and electrolyte disorders; or uncontrolled seizures).

Liver Effects: As with other HMG-CoA reductase inhibitors, rosuvastatin should be used with caution in patients who consume excessive quantities of alcohol and/or have a history of liver disease.

It is recommended that liver function tests be carried out prior to, and 3 months following, the initiation of treatment. Rosuvastatin should be discontinued or the dose reduced if the level of serum transaminases is greater than 3 times the upper limit of normal.

In patients with secondary hypercholesterolaemia caused by hypothyroidism or nephrotic syndrome, the underlying disease should be treated prior to initiating therapy with rosuvastatin

Rosuvastatin should discontinued or the dose reduced if the level of serum transaminases is greater than normal. The reporting rate for serious hepatic events (consisting mainly of increased hepatic transaminases) is higher at the 40mg dose.

Race: Pharmacokinetic studies show an increase in exposure in Asian subjects compared with Caucasians.

Myasthenia Gravis/Ocular Myasthenia: In few cases, statins have been reported to induce de novo or aggravate pre-existing myasthenia gravis or ocular myasthenia. Rosuvuid tablet should be discontinued in case of aggravation of symptoms. Recurrences when the same or a different statin was (re-) administered have been reported.

Protease inhibitors: Increased systemic exposure to rosuvastatin has been observed in subjects receiving rosuvastatin concomitantly with various protease inhibitors in combination with ritonavir. Consideration should be given both to the benefit of lipid lowering by use of rosuvastatin in HIV patients receiving protease inhibitors and the potential for increased rosuvastatin plasma concentrations when initiating and up titrating rosuvastatin doses in patients treated with protease inhibitors. The concomitant use with certain protease inhibitors is not recommended unless the dose of rosuvastatin is adjusted.

Lactose intolerance: Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

Interstitial lung disease: Exceptional cases of interstitial lung disease have been reported with some statins, especially with long term therapy. Presenting features can include dyspnoea, non-productive cough and deterioration in general health (fatigue, weight loss and fever). If it is suspected a patient has developed interstitial lung disease, statin therapy should be discontinued.

Diabetes Mellitus: Some evidence suggests that statins as a class raise blood glucose and in some patients, at high risk of future diabetes, may produce a level of hyperglycaemia where formal diabetes care is appropriate. This risk, however, is outweighed by the reduction in vascular risk with statins and therefore should not be a reason for stopping statin treatment. Patients at risk (fasting glucose 5.6 to 6.9 mmol/l, BMI >30 kg/m², raised triglycerides, hypertension) should be monitored both clinically and biochemically according to national guidelines.

Pediatric patients (10 to 17 years of age): The evaluation of linear growth (height), weight, BMI (body mass index) and secondary characteristics of sexual maturation by Tanner staging in pediatric patients taking rosuvastatin is limited to a one year period.

PREGNANCY AND LACTATION

Rosuvastatin is contraindicated in pregnancy and lactation. Women of child bearing potential should use appropriate contraceptive measures. Since cholesterol and other products of cholesterol biosynthesis are essential for the development of the foetus, the potential risk from inhibition of HMG-CoA reductase outweighs the advantage of treatment during pregnancy.

If a patient becomes pregnant during use of this product, treatment should be discontinued immediately. There are no data with respect to excretion in milk in humans.

MAIN SIDE / ADVERSE EFFECTS

The adverse reactions seen with rosuvastatin are generally mild and transient. Less than 4% of rosuvastatin-treated patients were withdrawn due to adverse reactions.

The frequencies of adverse reactions are ranked according to the following convention: Common; Uncommon; Rare; Very rare; Not known.

Blood and lymphatic system disorders

Rare: Thrombocytopenia

Immune system disorders

Rare: Hypersensitivity reactions including angioedema

Endocrine disorders

Common: Diabetes mellitus

Eye disorders

Not known: Ocular myasthenia

Psychiatric disorders

Not known: Depression

Nervous system disorders

Common: Headache dizziness

Very rare: Polyneuropathy, memory loss

Not known: Peripheral neuropathy, sleep disturbances (including insomnia and nightmares) Myasthenia gravis

Respiratory, thoracic and mediastinal disorders

Not known: Cough, dyspnea

Gastro-intestinal disorders

Common: Constipation; Nausea, abdominal pain

Rare: Pancreatitis

Not known: Diarrhoea

Hepatobiliary disorders

Rare: Increased hepatic transaminases

Very rare: Jaundice, hepatitis

Skin and subcutaneous tissue disorders

Uncommon: Pruritis, rash, urticaria

Not known: Stevens-Johnson syndrome

- Musculo-skeletal and connective tissue disorders:**
Common: Myalgia
Rare: Myopathy (including myositis), rhabdomyolysis
Very rare: Arthralgia
Not known: Tendon disorders, sometimes complicated by rupture; Immune-mediated necrotizing myopathy
- Renal and urinary disorders**
Very rare: Haematuria
- Reproductive system and breast disorders**
Very rare: Gynaecomastia
- General disorders and administration site conditions**
Common: Asthenia
Not known: Oedema

Undesirable Effects:
There have been rare post-marketing reports of cognitive impairment (e.g. memory loss, forgetfulness, amnesia, memory impairment, confusion) associated with statin use. These cognitive issues have been reported for all statins. The reports are generally non-serious and reversible upon statin discontinuation, with variable times to symptom onset (1 day to years) and symptom resolution (median 3 weeks). Increases in HbA1c and fasting blood glucose have been reported with statins. The risk of hyperglycemia, however, is outweighed by the reduction in vascular risk with statins.

DRUG INTERACTIONS

Effect of co-administered medicinal products on rosuvastatin
Transporter protein inhibitors: Rosuvastatin is a substrate for certain transporter proteins including the hepatic uptake transporter OATP1B1 and efflux transporter BCRP. Concomitant administration of rosuvastatin with medicinal products that are inhibitors of these transporter proteins may result in increased rosuvastatin plasma concentrations and an increased risk of myopathy.
- **Ciclosporin:** During concomitant treatment with rosuvastatin and ciclosporin, rosuvastatin AUC values were on average 7 times higher than those observed in healthy volunteers. Rosuvastatin is contraindicated in patients receiving concomitant ciclosporin. Concomitant administration did not affect plasma concentrations of ciclosporin.
- **Protease inhibitors:** Although the exact mechanism of interaction is unknown, concomitant protease inhibitor use may strongly increase rosuvastatin exposure. The concomitant use of rosuvastatin and some protease inhibitor combinations may be considered after careful consideration of rosuvastatin dose adjustments based on the expected increase in rosuvastatin exposure.

Gemfibrozil and other lipid-lowering products: Concomitant use of rosuvastatin and gemfibrozil resulted in a 2-fold increase in rosuvastatin C_{max} and AUC.

Based on data from specific interaction studies no pharmacokinetic relevant interaction with fenofibrate is expected, however a pharmacodynamic interaction may occur.

Gemfibrozil, fenofibrate, other fibrates and lipid lowering doses (> or equal to 1g/day) of niacin (nicotinic acid) increase the risk of myopathy when given concomitantly with HMG-CoA reductase inhibitors, probably because they can produce myopathy when given alone.

Antacid: The simultaneous dosing of rosuvastatin with an antacid suspension containing aluminium and magnesium hydroxide resulted in a decrease in rosuvastatin plasma concentration of approximately 50%. This effect was mitigated when the antacid was dosed 2 hours after rosuvastatin.

Erythromycin: Concomitant use of rosuvastatin and erythromycin resulted in a 20% decrease in AUC and a 30% decrease in C_{max} of rosuvastatin. This interaction may be caused by the increase in gut motility caused by erythromycin.

Cytochrome P450 enzymes: Rosuvastatin is neither an inhibitor nor an inducer of cytochrome P450 isoenzymes. In addition, rosuvastatin is a poor substrate for these isoenzymes. Therefore, drug interactions resulting from cytochrome P450-mediated metabolism are not expected. No clinically relevant interactions have been observed between rosuvastatin and either fluconazole (an inhibitor of CYP2C9 and CYP3A4) or ketoconazole (an inhibitor of CYP2A6 and CYP3A4).

Fusidic Acid: Interaction studies with rosuvastatin and fusidic acid have not been conducted. As with other statins, muscle related events, including rhabdomyolysis, have been reported in post-marketing experience with rosuvastatin and fusidic acid given concurrently.

Therefore, the combination rosuvastatin and fusidic acid is not recommended. If possible, temporary suspension of rosuvastatin treatment is recommended. If unavoidable, patients should be closely monitored.

Ezetimibe: Concomitant use of 10 mg rosuvastatin and 10 mg ezetimibe resulted in a 1.2 fold increase in AUC of rosuvastatin in hypercholesterolaemic subjects. A pharmacodynamic interaction, in terms of adverse effects, between rosuvastatin and ezetimibe cannot be ruled out.

Interactions requiring rosuvastatin dose adjustments: When it is necessary to co-administer Rosuvid with other medicinal products known to increase exposure to rosuvastatin, doses of Rosuvid should be adjusted. Start with a 5 mg once daily dose of Rosuvid if the expected increase in exposure (AUC) is approximately 2-fold or higher.

The maximum daily dose of Rosuvid should be adjusted so that the expected rosuvastatin exposure would not likely exceed that of a 40 mg daily dose of Rosuvid taken without interacting medicinal products, for example a 20 mg dose of Rosuvid with gemfibrozil (1.9-fold increase), and a 10 mg dose of Rosuvid with combination ritonavir/atazanavir (3.1-fold increase).

Example of medicinal products that will interact with Rosuvid and thus affect the rosuvastatin exposure: Ciclosporin, Atazanavir, Lopinavir, Gemfibrozil, Eltrombopag, Darunavir, Tipranavir, Dronedarone, Itraconazole, Ezetimibe, Fosamprenavir, Alectiazar, Silymarin, Fenofibrate, Rifampin, Ketoconazole, Fluconazole, Erythromycin, Clopidogrel, Simeprevir and Baicalin.

Effect of rosuvastatin on co-administered medicinal products

Vitamin K antagonists: As with other HMG-CoA reductase inhibitors, the initiation of treatment or dosage up-titration of rosuvastatin in patients treated concomitantly with vitamin K antagonists (e.g. warfarin or another coumarin anticoagulant) may result in an increase in International Normalised Ratio (INR). Discontinuation or down-titration of rosuvastatin may result in a decrease in INR. In such situations, appropriate monitoring of INR is desirable.

Oral contraceptive/hormone replacement therapy (HRT): Concomitant use of Rosuvastatin and an oral contraceptive resulted in an increase in ethinyl estradiol and norgestrel AUC of 26% and 34%, respectively. These increased plasma levels should be considered when selecting oral contraceptive doses. There are no pharmacokinetic data available in subjects taking concomitant rosuvastatin and HRT and therefore a similar effect cannot be excluded.

Other medicinal products: Based on data from specific interaction studies no clinically relevant interaction with digoxin is expected.

Paediatric population: Interaction studies have only been performed in adults. The extent of interactions in the paediatric population is not known.

Other medications: Concurrent use of fibrates may cause severe myositis and myoglobinuria.

OVERDOSE AND TREATMENT

There is no specific treatment in the event of overdose. In the event of overdose, the patient should be treated symptomatically and supportive measures instituted as required. Liver function and CK levels should be monitored. Haemodialysis is unlikely to be of benefit.

DOSAGE AND ADMINISTRATION

For oral use.

Recommended Dose

Patients should be placed on a standard cholesterol-lowering diet (at least equivalent to the Adult Treatment Panel III (ATP III TLC diet)) before receiving Rosuvid Tablet, and should continue on this diet during treatment with Rosuvid Tablet. If appropriate, a program of weight control and physical exercise should be implemented.

Prior to initiating therapy with Rosuvid Tablet, secondary causes for elevations in plasma lipid levels should be excluded. A lipid profile should also be performed. After initiation or upon titration of Rosuvid Tablet, lipid levels should be analyzed within 2-4 weeks and the dosage adjusted accordingly.

Rosuvid Tablet

The usual recommended starting dose of Rosuvid Tablet is 10 mg once daily. However, initiation of therapy with 5 mg once daily should be considered for special patient populations or patients requiring less aggressive LDL-C reductions. The choice of starting dose should take into account the individual patients' cholesterol level and future cardiovascular risk as well as the potential risk for adverse reactions. Rosuvid Tablet may be taken in the morning or evening, with or without food. The majority of patients are controlled at the 10mg dose. However, if necessary, dose adjustments to the next dose level can be made after 4-week intervals. The maximum response is usually achieved within 2-4 weeks and is maintained during chronic therapy. Increasing the dose to 40 mg should be reserved for patients with severe hypercholesterolaemia at high cardiovascular risk (in particular those with familial hypercholesterolaemia), who do not achieve their treatment goal on 20 mg and should only be initiated under specialist supervision (see Warnings and Precautions).

The physician who selects to use Rosuvid Tablet at a dose higher than 20 mg should periodically re-evaluate the long term risk/benefit of Rosuvid Tablet for the individual patient. Rosuvid Tablet should be prescribed with caution in patients with pre-disposing factors for myopathy / rhabdomyolysis (see Warnings and Precautions).

The dosage of Rosuvid Tablet should be individualised according to baseline LDL-C, total-C/HDL-C ratio and/or TG levels, the recommended target lipid values (see Recommendations for the Management and Treatment of Dyslipidemia [Canada] summarised below in Table 1) and/or the Third Report of the U.S. National Cholesterol Education Program [NCEP Adult Treatment Panel III]) and the patient response.

Lipid levels should be monitored periodically and, if necessary, the dose of Rosuvid Tablet adjusted based on target lipid levels recommended by guidelines.

Table 1: Canadian Recommendations for Target Lipid Values Based on Level of Risk

Level of Risk (definition)	Target values LDL-C (mmol/L)	Total-C/ HDL-C ratio	TG (mmol/L)
Very high* (10-year risk of CAD>30%, or history of cardiovascular disease or diabetes)	<2.5	<4.0	<2.0
High* (10-year risk of CAD 20% - 30%)	<3.0	<5.0	<2.0
Moderate** (10-year risk of CAD 10% - 20%)	<4.0	<6.0	<2.0
Low*** (10-year risk of CAD <10%)	<5.0	<7.0	<3.0

- * Start medication and lifestyle changes concomitantly if values are above target values.
- ** Start medication if target values are not achieved after 3 months of lifestyle modification.
- *** Start medication if target values are not achieved after 6 months of lifestyle modification.

Dosage in patients with renal insufficiency: The usual dose range applies in patients with mild to moderate renal impairment. The use of Rosuvid Tablet in patients with severe renal impairment is contraindicated.

Dosage in patients with hepatic insufficiency: Rosuvastatin is contraindicated in patients with active liver disease.

Use in the elderly: The efficacy of rosuvastatin in the geriatric population (≥65 years of age) was comparable to the efficacy observed in the non-elderly.

Pediatric Patients (10 to 17 years of age): In pediatric patients (10 to 17 years of age) with heterozygous familial hypercholesterolemia the usual dose range of Rosuvid Tablet is 5-20 mg/day; the maximum recommended dose is 20 mg/day (doses greater than 20 mg have not been studied in this patient population). Doses should be individualized according to the recommended goal of therapy (see Indications). Adjustments should be made at intervals of 4 weeks or more.

Use in Children below 10 years: The safety and effectiveness in children have not been established. In children and adolescents with homozygous familial hypercholesterolemia experience is limited to eight patients (aged 8 years and above).

Dosage on Asian Patients: Initiation of Rosuvid Tablet therapy with 5 mg once daily should be considered for Asian patients. The potential for increased systemic exposures relative to Caucasians is relevant when considering escalation of dose in cases where hypercholesterolaemia is not adequately controlled at doses of 5, 10 or 20 mg once daily (see Warnings and Precautions and Pharmacokinetic).

Genetic polymorphisms: Specific types of genetic polymorphisms are known that can lead to increased rosuvastatin exposure (see Pharmacokinetic). For patients who are known to have such specific types of polymorphisms, a lower daily dose of Rosuvid Tablet is recommended.

Dosage in patients with pre-disposing factors to myopathy: The recommended start dose is 5 mg in patients with predisposing factors to myopathy (see Warnings and Precautions).

Concomitant therapy: Rosuvastatin is a substrate of various transporter proteins (e.g. OATP1B1 and BCRP). The risk of myopathy (including rhabdomyolysis) is increased when Rosuvid Tablet is administered concomitantly with certain medicinal products that may increase the plasma concentration of rosuvastatin due to interactions with these transporter proteins (e.g. ciclosporin and certain protease inhibitors including combinations of ritonavir with atazanavir, lopinavir, and/or tipranavir; see Warnings and Precautions for use and interaction with other medicinal products and other forms of interaction). Whenever possible, alternative medications should be considered, and, if necessary, consider temporarily discontinuing Rosuvid Tablet therapy. In situations where co-administration of these medicinal products with Rosuvid Tablet is unavoidable, the benefit and the risk of concurrent treatment and Rosuvid Tablet dosing adjustments should be carefully considered (see Interactions With Other Medicaments).

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage:
Store below 30°C. Protect from light and moisture.

Presentation/ Packing:
Alu-Alu Blister of 1 x 10's, 3 x 10's and 10 x 10's.

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